

UnitedHealth Group Incorporated

Trough Earnings, Trough Multiple and a 2026 Rate Card Already Written: Overweight Into the Reset, Dec-26 PT \$320

UNH, NYSE | Managed Care

OVERWEIGHT

Price (21 Jul 2025): **\$282.14**
Price Target (Dec-26): **\$320.00**

INVESTMENT THESIS

We resume coverage of UnitedHealth at Overweight with a Dec-26 target of \$320, a total return of ~16.6% against the \$282.14 close of 21 July (+13.4% on price, +3.1% from the declared \$8.84 dividend). The debate is not whether 2025 is a bad year; it is whether the Medicare Advantage and Optum Health cost shock is a one-year repricing problem or a permanent reset of the earnings base. Our answer is mostly the former, held with less conviction than we would like. FY25 adjusted EPS guidance was cut to \$26.00-26.50 from \$29.50-30.00 on 17 April, the shares fell 22% in a day, and on 13 May the outlook was withdrawn altogether and Stephen Hemsley replaced Andrew Witty. Ten weeks on, the largest healthcare company in the United States is still not telling investors what it earns.

What is knowable is that 2026 is already repriced on the revenue side. CMS finalised a 5.06% expected average change in Medicare Advantage revenue on 7 April against 3.70% for 2025, on an effective growth rate of 9.04% versus 2.33%; the 2024 CMS-HCC risk model reaches full phase-in in 2026; and bids carrying the elevated trend were filed on 2 June. Against that sit a medical care ratio up from 82.0% in FY22 to 85.5% in FY24 and 84.8% in Q1'25, days claims payable down every year over the same span from 49.9 to 45.5, and Optum Health operating earnings guided to \$6.2-6.4bn from \$7.8bn. At 12.8x a rebased FY26E and 10.2x what the company actually earned last year, the price embeds a permanent impairment. We do not think it is permanent. We do think the 29 July print is the risk.

- 2026 is repriced on both sides: CMS at +5.06% against +3.70% for 2025, bids filed 2 June with elevated trend in, risk-model phase-in complete.
- The reserve cushion is thinning: days claims payable 45.5 at Q1'25 against 49.9 at FY22, medical care ratio up 280bp over the same stretch.
- Honestly a close call: the Overweight needs FY26E near \$22.00. At \$19.80 the same method gives \$306 and a Neutral.

COMPANY OVERVIEW

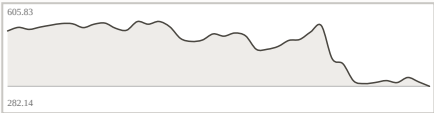
UnitedHealth is the largest US health carrier and, through Optum, the largest employer of physicians in the country. UnitedHealthcare (\$84.6bn of Q1'25 revenue, +12% YoY) writes risk and administrative-services-only benefits for 50.1m medical members: 8.2m Medicare Advantage, 7.6m Medicaid, 8.4m commercial risk and 21.6m commercial fee-based.

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PRICE PERFORMANCE



Actual daily closes, 22 Jul 2024 to 21 Jul 2025, sampled to 40 points. Intraday 52-week range \$248.88-\$630.73; last close \$282.14.

COMPANY DATA

Shares O/S (mm)	907.1
Market Cap (\$bn)	255.9
Enterprise Value (\$bn)	302.9
52-Week Range	\$248.88 - \$630.73
52-Week Price Change	-49.5%
Avg. Daily Volume (20d, mm)	11.7
Realised Vol. (1Y, ann.)	46.9%
Short Interest (% of S/O)	1.44%
Beta (5Y)	-
P/E (FY24A adj. GAAP)	10.2x 18.2x
P/E (FY25E FY26E)	14.5x 12.8x
Dividend (annualised)	\$8.84
Dividend Yield	3.13%
Fiscal Year End	December

KEY METRICS / ESTIMATES

(\$bn, except EPS)	FY23A	FY24A	FY25E	FY26E
Revenues	371.6	400.3	-	-
Earnings from ops.	32.4	32.3	-	-
Adjusted EPS	\$25.12	\$27.66	\$19.50	\$22.00
% growth	13.2%	10.1%	-30%	+13%
GAAP diluted EPS	\$23.86	\$15.51	-	-
Medical care ratio	83.2%	85.5%	-	-
Days claims payable	47.9	47.0	-	-
P/E on adj. EPS	11.2x	10.2x	14.5x	12.8x

FY23A/FY24A per company results releases (12 Jan 2024, 16 Jan 2025; SEC EDGAR). UnitedHealth has no standing guidance: the FY25 outlook was cut on 17 Apr 2025 and withdrawn in full on 13 May 2025. FY25E of \$19.50 is analyst-derived, the midpoint of the only two firm-specific published estimates datable to on or before 22 Jul 2025 (Wolfe Research \$18.00, 10 Jul; Barclays \$20.00-21.00, 9 Jul). FY26E of \$22.00 is Wolfe Research's estimate of 10 Jul 2025, the only datable 2026 figure. Aggregator consensus is deliberately not used: no snapshot could be dated to on or before 22 Jul 2025. Further sourcing detail is set out under Important Disclosures.

Optum (\$63.9bn) is three businesses: Optum Health (\$25.3bn, -5% YoY), care delivery that is largely value-based and capitated on Medicare lives; Optum Rx (\$35.1bn, +14%), the pharmacy benefit manager, 1.62bn adjusted scripts in FY24; and Optum Insight (\$4.6bn, +2%), data and revenue-cycle services on a \$32.9bn backlog. Optum contributed \$16.7bn of FY24 operating earnings against UnitedHealthcare's \$15.6bn: the group is no longer principally an insurer.

Q1 2025 RESULTS AND THE WITHDRAWN OUTLOOK

Q1'25, reported 17 April, produced the worst single day in the stock since 1998. Revenue of \$109.6bn grew 9.8% but came in roughly \$2bn light of the Street, and adjusted EPS of \$7.20 (GAAP \$6.85) missed by seven cents. The absolute numbers were not the problem: earnings from operations of \$9.1bn were up from \$7.9bn and operating cash flow was \$5.5bn against \$1.1bn in a cyberattack-distorted base. The problem was the guide, with FY25 adjusted EPS cut to \$26.00-26.50 from \$29.50-30.00 one quarter into the year and the FY25 medical care ratio reset to about 87.5% against 85.5% in FY24.

Andrew Witty gave two reasons and both matter. On Medicare Advantage: "we had planned for 2025 care activity to increase at a rate consistent with the utilization trend we saw in 2024. Instead though, first quarter 2025 indications suggest care activity increased at twice that rate." On Optum Health: "we added more new Medicare patients to Optum Health, a portion of whom were covered by plans that were exiting markets", patients who arrived with unmeasured acuity into a capitated economic model calibrated on a prior risk-score basis. Optum Health FY25 operating earnings were guided to \$6.2-6.4bn against \$7.8bn delivered in FY24. Witty called the quarter "unusual and unacceptable" and argued the drivers were "highly addressable as we look ahead to 2026". Chief Financial Officer John Rex was blunter: "it's an outlook that I'm extremely disappointed to share with you."

The 13 May announcement was the more consequential event. UnitedHealth suspended the FY25 outlook entirely, citing care activity accelerating across a broader set of benefit types, medical costs of Medicare Advantage members new to UnitedHealthcare running above plan, and further pressure at Optum Health. Witty stepped down for personal reasons after four years; Stephen Hemsley, chief executive from 2006 to 2017 and chairman since, took the role back with immediate effect and Witty stayed as senior adviser. The company said it expected to return to growth in 2026. The shares fell 18% on the day and a further 11% on 15 May after the Wall Street Journal reported a Department of Justice criminal investigation into the Medicare Advantage business, which UnitedHealth said it had "not been notified" of and described as "deeply irresponsible" reporting.

Withdrawing an outlook rather than cutting it a second time is a specific signal, and it is not a good one. It says the range could not be bounded. Hemsley set the calendar himself at the 2 June annual meeting: "with our second quarter report on July 29, we will establish a prudent 2025 earnings outlook and offer initial perspectives for 2026." Everything in this note is written one week before that. We are underwriting a franchise and a rate card, not a guidance range, because on 22 July there is no guidance range to underwrite.

FINANCIAL SUMMARY

UnitedHealth Group Incorporated -- Summary Financials

\$ in billions, except per-share data	FY23A	FY24A	Q1'25A	FY25E	FY26E
Total revenues	371.6	400.3	109.6	-	-
% growth YoY	14.6%	7.7%	9.8%	-	-
UnitedHealthcare	281.4	298.2	84.6	-	-
Optum Health	95.3	105.4	25.3	-	-
Optum Insight	18.9	18.8	4.6	-	-
Optum Rx	116.1	133.2	35.1	-	-
Earnings from operations	32.4	32.3	9.1	-	-
UnitedHealthcare	16.4	15.6	5.2	-	-
Optum Health	6.6	7.8	1.6	6.2-6.4	-
Optum Insight	4.3	3.1	1.0	-	-
Optum Rx	5.1	5.8	1.3	-	-
Medical care ratio	83.2%	85.5%	84.8%	~87.5%	-
Operating cost ratio	14.7%	13.2%	12.4%	-	-
Days claims payable	47.9	47.0	45.5	-	-
Cash flow from operations	29.1	24.2	5.5	-	-
Diluted EPS (GAAP)	\$23.86	\$15.51	\$6.85	-	-
Adjusted EPS	\$25.12	\$27.66	\$7.20	\$19.50	\$22.00
P/E on adj. EPS (@ \$282.14)	11.2x	10.2x	-	14.5x	12.8x

Memo: FY25 revenue guidance of \$450-455bn and adjusted EPS guidance of \$29.50-30.00, issued 4 Dec 2024 and affirmed 16 Jan 2025, were cut to \$26.00-26.50 adjusted EPS on 17 Apr 2025 and withdrawn in full on 13 May 2025.

Source: UnitedHealth Group results releases of 12 Jan 2024 (FY23), 16 Jan 2025 (FY24) and 17 Apr 2025 (Q1'25), and the Q1 2025 Form 10-Q, all via SEC EDGAR and the company investor site; Akul Veauli Securities LLC. FY22 comparatives referenced elsewhere in this note (medical care ratio 82.0%, days claims payable 49.9, adjusted EPS \$22.19) are per the 12 Jan 2024 release. Optum Insight Q1'25 earnings from operations were \$961mm, shown rounded. The FY25E Optum Health earnings range and the ~87.5% FY25E medical care ratio are the only surviving elements of the 17 Apr 2025 guidance we cite, and both formed part of the outlook withdrawn on 13 May; they are shown as the company's last stated view, not as our estimate. FY25E and FY26E adjusted EPS are analyst-derived and single-source respectively, as set out in the sidebar footnote and the Valuation section. All other FY25E/FY26E cells are "-" because UnitedHealth has no standing guidance and we could not date an aggregator consensus snapshot to on or before 22 Jul 2025. Multiples computed on the \$282.14 close of 21 Jul 2025.

THE COST TREND AND THE RESERVE CUSHION

The bear case does not need the Department of Justice. It needs two lines of the disclosure, and both are going the wrong way at once. The medical care ratio has risen from 82.0% in FY22 to 83.2%, then 85.5% in FY24, with Q1'25 at 84.8% against 84.3% a year earlier and the withdrawn FY25 guide at about 87.5%. That is roughly 550bp of gross margin surrendered in three years on a revenue base of \$400bn. Every 100bp of medical care ratio is worth on the order of \$2.5bn of pre-tax earnings at UnitedHealthcare's current risk revenue, so the arithmetic of the FY25 collapse is almost entirely explained by this one line.

The second line is days claims payable, and it is the one we would put in front of any investor who thinks the reset is finished. It has fallen in every period on the tape: 49.9 at FY22, 47.9 at FY23, 47.0 at FY24 and 45.5 at Q1'25, against 47.1 in Q1'24. A carrier reducing its claims reserve cushion while telling investors that care activity is running at twice the assumed rate is running two exposures in the same direction. If the reserve is adequate, the ratio simply reflects faster claims processing, which is the company's standing explanation. If it is not, prior-period development turns negative and the medical care ratio gets worse before it gets better. We cannot resolve that from outside, and we say so: it is the reason our target carries a 20% weight on an anchor that does not depend on earnings at all.

What we can observe is that this is not idiosyncratic. Centene withdrew its own 2025 guidance on 1 July, Molina cut its outlook in the same week, and Elevance cut FY25 adjusted EPS to about \$30 from at least \$34.15 on 17 July, its shares falling roughly 11%, citing exchange and Medicaid acuity, behavioural health, specialty pharmacy and aggressive provider billing. Sector-wide trend is a cyclical fact that gets repriced in bids. Company-specific execution failure is not. On the evidence available today, UnitedHealth's problem is roughly three parts the former to one part the latter, and it is the second part, concentrated in Optum Health, that we are least comfortable underwriting.

WHAT 2026 IS ALREADY PRICED TO PAY

The single strongest fact available on 22 July is that the 2026 revenue side is not a forecast. CMS published the CY2026 Rate Announcement on 7 April with an expected average change in revenue of 5.06%, over \$25bn of incremental industry funding, against 3.70% for CY2025. The underlying effective growth rate is 9.04% versus 2.33% a year earlier, a swing driven by fee-for-service cost experience that is now finally catching up with the utilization the plans have been absorbing. That figure was also a large upgrade from the 2.23% proposed in the Advance Notice in January, so the final number surprised to the upside inside a quarter in which everything else about the sector surprised to the downside.

Three further mechanics compound in the same direction. First, 2026 is the terminal year of the three-year phase-in of the 2024 CMS-HCC risk model: from that point 100% of risk scores are calculated on the new basis, and the incremental coding step that has run against risk-adjusted revenue since 2024 goes to zero thereafter. The 2026 announcement still carries a combined 3.01% drag from the risk model revision and fee-for-service normalisation, so the transition is not costless next year, but it is finite and it is the last instalment. Second, the bids that set 2026 benefit design, county footprint and margin were filed on 2 June, and Hemsley confirmed the point directly: "we have incorporated our elevated care activity experience into the Medicare Advantage bids submitted to CMS this week." Third, the levers available to a carrier that has priced badly are blunt but effective. Benefits get cut, counties get exited, and members leave. UnitedHealth has run that play before and it works on margin even when it hurts on membership.

The honest qualification is that none of this touches Optum Health, which is where we are least confident. Optum Health revenue fell year over year in Q1, to \$25.3bn from \$26.7bn, with operating earnings of \$1.6bn against \$1.9bn and margin of 6.3% against 7.1%. A capitated care-delivery business whose revenue is shrinking while its patient panel grows is a business whose unit economics are not yet under control. Hemsley's own characterisation on 2 June was that Optum Health "is in its formative stages" and requires "greater operational rigor and maturity" to produce "more reliable views of future performance". We take him at his word, which is precisely the problem: that is the language of a multi-year rebuild, not a one-year rebase, and it is why we underwrite the UnitedHealthcare repricing story and treat Optum Health recovery as option value rather than base case.

VALUATION AND PRICE TARGET

Our \$320 Dec-26 target is a 50/30/20 blend of three anchors, each computed against the \$282.14 close of 21 July. (1) Forward earnings, 50%: 14.5x applied to FY26E adjusted EPS of \$22.00 gives \$319.00. That multiple is the multiple the stock itself carries today on our FY25E of \$19.50, so the anchor embeds no re-rating whatsoever and is funded entirely by one year of earnings recovery; it is also a 24% discount to the 19.2x average at which UNH traded in each of the last three Decembers against the company's own initial guidance for the following year (21.5x on \$530.18 and \$24.65, 19.0x on \$526.47 and \$27.75, 17.0x on \$505.86 and \$29.75). Sensitivity: 13.0x gives \$286.00 and 16.0x gives \$352.00. (2) Published targets, 30%: the mean of the eleven most recent standing Street targets we can date on or before 21 July is \$345.45, median \$342, spanning \$270 (HSBC, 21 May) to \$418 (J.P. Morgan, 20 June). (3) Dividend support, 20%: the \$2.21 quarterly dividend declared on 4 June is an \$8.84 annual rate yielding 3.13% on the close, the widest in the company's history; capitalising that unchanged rate at a 3.00% yield gives \$294.67. The blend is \$322.07, rounded down to \$320.

UnitedHealth Group -- price target derivation

Anchor	Weight	Input	Output
Forward earnings: 14.5x FY26E adjusted EPS	50%	\$22.00	\$319.00
Published Street targets, 11 firms dated on or before 21 Jul 2025	30%	mean \$345.45	\$345.45
Dividend support: declared rate at a 3.00% yield	20%	\$8.84	\$294.67
Weighted blend	100%	-	\$322.07
Price target (Dec-26, rounded)	-	-	\$320.00
Implied total return vs \$282.14 close	-	+13.4% price, +3.1% dividend	+16.6%

Source: Akul Veauli Securities LLC; company disclosure; broker actions as reported by public aggregators. Standing targets used in anchor 2, each a firm's most recent published action on or before 21 Jul 2025: TD Securities \$308 (19 May), HSBC \$270 (21 May), KeyBanc \$400 (2 Jun), Piper Sandler \$353 (3 Jun), Baird \$312 (11 Jun), J.P. Morgan \$418 (20 Jun), UBS \$385 (25 Jun), Barclays \$337 (9 Jul), Wolfe Research \$330 (10 Jul), Morgan Stanley \$342 (13 Jul), Truist \$345 (16 Jul). Historical multiples in anchor 1 use UnitedHealth's year-end closes of 30 Dec 2022, 29 Dec 2023 and 31 Dec 2024 against the midpoints of the initial next-year adjusted EPS guidance issued on 28 Nov 2022, 28 Nov 2023 and 4 Dec 2024. Total return assumes the declared \$8.84 annual dividend is received unchanged and is not reinvested.

We want the reader to see how narrow this is. The rating flips to Neutral at any target below \$315.62, which is 1.4% below the computed blend, and it also flips on either of two single-variable changes. If FY26E is \$19.80 rather than \$22.00, which is Wolfe Research's \$18.00 for 2025 grown 10%, anchor 1 falls to \$287.10 and the blend to \$306.12, a +11.6% total return and a Neutral. If the target multiple is 13.0x rather than 14.5x, the blend is \$305.57 and the conclusion is identical. An analyst who takes the low end of the datable estimate range, or who declines to pay 14.5x for a company whose own board withdrew its guidance ten weeks ago, lands at Neutral. That is a respectable position. We hold ours, and we hold it by 1.6 percentage points.

The estimates carry a health warning we would rather state than bury. UnitedHealth has no standing guidance, so there is no anchor to lean on. Our FY25E of \$19.50 is the midpoint of the only two firm-specific published views we could date on or before 22 July: Wolfe Research at \$18.00 on 10 July and Barclays on 9 July expecting a reinstated guide of \$20.00-21.00, down 26% year over year. A 15% spread between two houses one week before the print is not a consensus, it is the absence of one, and that dispersion is a large part of why the shares trade where they do. Our FY26E of \$22.00 is Wolfe's, the only datable firm-specific 2026 figure we found. We have deliberately not used aggregator consensus for either year because we could not date any snapshot to on or before 22 July, and an undated consensus in a name whose guidance was pulled in May is worse than no consensus at all. At our target, UNH would trade on 14.5x FY26E and 16.4x FY25E adjusted EPS, a 2.76% dividend yield and a market capitalisation of \$290bn against \$256bn today.

UPSIDE RISKS

Developments that could drive the shares above our price target include, but are not limited to, the following:

- The 2026 rate card is legislated, not forecast. CMS finalised a 5.06% expected average change in Medicare Advantage revenue on 7 April against 3.70% for 2025, on an effective growth rate of 9.04% versus 2.33%. That is over \$25bn of incremental industry funding already written into next year's revenue line before any management action.
- The risk-model transition ends. 2026 is the final instalment of the three-year phase-in of the 2024 CMS-HCC model; thereafter the incremental coding step is zero, removing a headwind that has run against risk-adjusted revenue in each of 2024, 2025 and 2026.
- Bids are already repriced. Hemsley confirmed on 2 June that "we have incorporated our elevated care activity experience into the Medicare Advantage bids submitted to CMS this week." Benefit design and county footprint are the levers, and a carrier willing to shed members can restore margin quickly.
- A prudent reinstatement is a clearing event. UBS made exactly that argument on 25 June in maintaining a Buy. A guidance number the market believes is worth more than a higher number it does not, and the 29 July print is the first opportunity in ten weeks to supply one.
- Trough multiple on trough earnings. At \$282.14 the shares trade at 10.2x the \$27.66 adjusted EPS the company actually delivered in FY24 and 0.64x FY24 revenue. Any FY26 outcome above \$22.00 at an unchanged 14.5x prints above our target.
- Optum Health's shortfall is largely attributable. The \$1.4-1.6bn gap between the \$6.2-6.4bn FY25 guide and \$7.8bn delivered in FY24 was attributed to patients acquired from exiting plans and to the risk-model transition rather than to the underlying care model; a normal 2026 intake cohort removes most of it.
- Two of four segments are not part of the problem. Optum Insight Q1 operating earnings nearly doubled to \$961mm as the cyberattack comparison lapped, on a \$32.9bn backlog, and Optum Rx grew revenue 14% to \$35.1bn with operating earnings up to \$1.3bn.
- Capital return continues through the reset. The \$2.21 quarterly dividend declared on 4 June is a 3.13% yield, and nearly \$5bn was returned in dividends and buybacks in Q1 alone, so share count falls even in a bad year.

DOWNSIDE RISKS

Developments that could drive the shares below our price target include, but are not limited to, the following:

- There is no guidance. Withdrawing an outlook on 13 May rather than cutting it a second time is management stating that it could not bound the range. Every forward number in this note, ours included, is an inference drawn from outside the company.
- The reserve cushion is thinning. Days claims payable fell from 49.9 at FY22 to 47.9, then 47.0, then 45.5 at Q1'25 against 47.1 a year earlier. A falling claims reserve into an accelerating trend is the classic setup for adverse prior-period development, and it is the most uncomfortable number in the disclosure.
- Trend has not been shown to have stopped. Witty told investors on 17 April that Q1 care activity ran at twice the assumed 2024 rate, and the 13 May withdrawal said it had broadened across more benefit types. Nothing public since then says otherwise.
- The last stated FY25 medical care ratio was about 87.5%, roughly 200bp above FY24 and 550bp above FY22, on a base where each 100bp is worth on the order of \$2.5bn pre-tax. That guide was itself withdrawn three weeks after it was given.
- The sector tape has been repriced twice this month. Centene withdrew 2025 guidance on 1 July, Molina cut in the same week, and Elevance cut FY25 adjusted EPS to about \$30 from at least \$34.15 on 17 July with its shares down roughly 11%. UnitedHealth reports into that.
- Department of Justice exposure is live and unresolved in both directions. The Wall Street Journal reported on 14 May that a criminal investigation of the Medicare Advantage business has been running since at least mid-2024; the company said it had "not been notified" and called the reporting "deeply irresponsible". A separate civil False Claims Act matter on risk-adjustment submissions continues, with a Special Master recommending summary judgment for the company in March 2025 and the government objecting. The pending \$3.3bn Amedisys acquisition also remains subject to the department's November 2024 suit to block it.
- Optum Health may be a rebuild rather than a rebase. Revenue fell year over year in Q1 to \$25.3bn from \$26.7bn with margin at 6.3% against 7.1%, and Hemsley described the business on 2 June as "in its formative stages" needing "greater operational rigor and maturity". That is multi-year language.
- Medicaid and the exchanges are deteriorating independently of Medicare. The reconciliation act signed on 4 July reduces federal Medicaid funding and tightens eligibility, and the enhanced ACA premium subsidies are scheduled to lapse at the end of 2025. Both shrink or sour risk pools UnitedHealthcare serves.
- Valuation offers less of a floor than the multiple implies. Goodwill of \$107.6bn and other intangibles of \$22.9bn exceed total equity of \$100.8bn, so tangible book value is negative and there is no asset support beneath the earnings multiple. If FY26 lands at \$19.00 and the market pays 12x, the shares are \$228.
- This is no longer a defensive holding. Realised volatility over the trailing twelve months is 46.9%, the 52-week range spans \$248.88 to \$630.73, and the stock has fallen 49.5% in a year. Position sizing is a material risk in its own right, and the 29 July print is seven days away.

ANALYST CERTIFICATION

I, Akul Veauli, hereby certify that the views expressed in this research report accurately reflect my personal views about the subject company and its securities. I also certify that no part of my compensation was, is, or will be directly or indirectly related to the specific recommendations or views expressed in this report. I am a real person and the sole author of this report. I am not a registered or certified research analyst: any "AC" analyst-certification mark previously shown next to my name was self-applied for layout purposes, is not a real credential, and has been removed. "Akul Veauli Securities LLC" is an invented firm name and is not a real or registered entity.

IMPORTANT DISCLOSURES

Estimate and market-data sourcing, continued from the cover page. Remaining FY25E/FY26E cells are "-" rather than estimated. Multiples on the \$282.14 close of 21 Jul 2025. Short interest is 13.08m shares at the 30 Jun 2025 settlement date (1.13 days to cover), the latest report published by 22 Jul. Beta is "-": no provider figure could be dated to the as-of window.

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Point-in-time basis: this note is written as of 22 July 2025 and uses no information published after that date. The quoted price is the 21 July 2025 close of \$282.14, the last close before the as-of date, and the 52-week range, average daily volume, realised volatility and short interest are all measured to on or before that date. UnitedHealth reported second-quarter 2025 results on 29 July 2025, one week after this note; nothing from that report, or from any subsequent company confirmation of investigations reported in the press, is reflected here.

Estimate sourcing: UnitedHealth withdrew its full-year 2025 outlook on 13 May 2025 and had no standing guidance as of 22 July 2025. FY2025E adjusted EPS of \$19.50 is analyst-derived as the midpoint of the two firm-specific published estimates datable to on or before 22 July 2025 (Wolfe Research \$18.00, 10 July; Barclays \$20.00-21.00, 9 July). FY2026E adjusted EPS of \$22.00 is Wolfe Research's published estimate of 10 July 2025. Published aggregator consensus has deliberately not been used because no snapshot could be dated to on or before the as-of date. All other forward-year cells in the Key Metrics and Financial Summary tables are shown as "-" rather than estimated. The FY2025 medical care ratio of ~87.5% and Optum Health earnings of \$6.2-6.4bn are the company's own 17 April 2025 guidance, subsequently withdrawn, and are labelled as such. Beta is shown as "-" because no provider figure could be dated to the as-of window. Short interest of 13,075,586 shares is per the 30 June 2025 settlement-date report, the most recent published on or before 22 July 2025.

The price performance chart is built from actual UnitedHealth daily closing prices between 22 July 2024 and 21 July 2025, sampled evenly to forty points and smoothed for rendering. It is therefore a reduced representation of a real series rather than a tick-accurate chart, and the axis labels reflect the sampled path rather than the intraday 52-week extremes disclosed in the Company Data block.

Nothing in this document constitutes investment advice, an offer, or a solicitation to buy or sell any security. No recipient should rely on any information in this document for any actual investment decision.

RATING DEFINITIONS

Overweight: total return expected to exceed the average total return of the analyst's coverage universe over the next 6-12 months. Neutral: total return expected to be broadly in line with that average. Underweight: total return expected to be below that average. For this note the rating was applied mechanically to the implied twelve-month total return from the derived price target, including the declared but not reinvested annual dividend: Overweight above +15%, Underweight below 0%, Neutral otherwise. UNH's implied total return of +16.6% (+13.4% price, +3.1% dividend) maps to Overweight; the rating would flip to Neutral at any price target below \$315.62. These are generic sell-side rating conventions, not a registered firm's published rating scale.

GENERAL DISCLAIMER

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